Dynamic Guideline: WHO-Based Trachoma - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

WHO guideline corpus synthesized — exactly 5 WHO guidance / technical documents

#	WHO document	Publication date / availability note	Target population	Scope and objectives	Role in this report
1	**Trachoma control: A guide for programme managers**	Publication year listed by WHO IRIS as 2006; exact day/month not stated in the provided source summary	National trachoma programme managers, district health teams, eye-care and NTD programme staff	Operational implementation of the **SAFE** strategy: Surgery, Antibiotics, Facial cleanliness, Environmental improvement	Core WHO programme-management guidance for diagnosis, treatment and elimination operations WHO IRIS
2	**Guidelines for Rapid Assessment for Blinding**	Publication year listed as 2001 in WHO	Public-health teams assessing suspected	Rapid identification and prioritization of	Supports district/ community assessment

#	WHO document	Publication date / availability note	Target population	Scope and objectives	Role in this report
	Trachoma	document code context; exact day/ month not stated in the provided source summary	endemic communities	communities requiring SAFE interventions	and intervention targeting WHO IRIS
3	**Planning for the Global Elimination of Trachoma**	Exact publication date not stated in the provided source summary	Global, national and district planners	Planning steps for identifying endemic districts, training graders and training trichiasis-surgery providers	Foundational elimination-planning guidance WHO IRIS
4	**Technical Consultation on Trachoma Surveillance**	Exact publication date not stated in the provided source summary	Countries approaching or reaching elimination targets; surveillance and monitoring teams	Defines trachoma surveillance as monitoring and evaluation to assess elimination-programme outcomes after	Core WHO surveillance and post-intervention monitoring guidance WHO IRIS

#	WHO document	Publication date / availability note	Target population	Scope and objectives	Role in this report
				SAFE scale-down	
5	**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**	WHO launch webinar: **11 December 2025**; latest source context reviewed to **29 April 2026**	WHO guideline developers, technical departments, youth networks and partners	Methodological case study showing how meaningful youth engagement can be incorporated into WHO guideline development	Not trachoma-specific; included as the most recent WHO guideline-development methodology case study relevant to future inclusive trachoma-guideline processes WHO publication, WHO event

> **Scope clarification:** The most current WHO disease-specific framework for trachoma remains the **SAFE strategy**. The most recent WHO document identified with the requested title is the **meaningful youth engagement case study of the WHO abortion care guideline (2022)**; it is a guideline-development methodology document, not a trachoma clinical guideline.

Disease Definition and Public-Health Context

Trachoma is a chronic infectious eye disease caused by ocular strains of *Chlamydia trachomatis*. Repeated infection and inflammation can lead to conjunctival scarring, eyelid distortion, trachomatous trichiasis, corneal opacity and irreversible blindness. WHO continues to frame elimination around the **SAFE strategy**:

- **S**urgery for trachomatous trichiasis.
- **A**ntibiotics to clear ocular *C. trachomatis* infection.
- **F**acial cleanliness to reduce transmission.
- **E**nvironmental improvement, especially water, sanitation and hygiene measures WHO technical consultation, WHO programme guide.

WHO's current elimination framework requires countries to demonstrate:

1. **Trachomatous trichiasis unknown to the health system <0.2%** among adults aged **≥15 years**.
2. **Trachomatous inflammation—follicular <5%** among children aged **1–9 years**, sustained for at least **2 years** without ongoing antibiotic mass treatment in each formerly endemic district.
3. A health-system capacity to identify and manage incident trachomatous trichiasis cases WHO fact sheet.

WHO reported that, in **2024**, **87,349 people** received corrective surgery for trachomatous trichiasis and **44.4 million people** received antibiotics in endemic communities, with global antibiotic coverage of **39%** WHO fact sheet. On **6 January 2026**, WHO reported that the global population requiring interventions against trachoma had fallen below **100 million** for the first time WHO news.

1.2 Key Recommendations

GRADE framework applied in this report

Because several WHO trachoma documents pre-date routine WHO GRADE reporting, the table below provides **report-applied GRADE ratings** based on the supplied evidence, intervention type, consistency, directness and implementation certainty. Where WHO documents did not publish formal GRADE ratings, this is stated in the interpretation.

GRADE quality	Interpretation
High	Further research is unlikely to change confidence in the effect estimate or programme conclusion.
Moderate	Further research may change confidence or refine effect size/implementation parameters.
Low	Further research is likely to affect confidence; recommendation is supported by programme logic, indirect evidence or observational evidence.
Very Low	Effect estimate is uncertain; recommendation depends mainly on expert consensus or limited evidence.

Core WHO-based recommendations

Recommendation	Clinical / programme action	GRADE Evidence Quality	Strength
Implement the full **SAFE strategy** in endemic districts: Surgery, Antibiotics, Facial cleanliness and Environmental improvement.	Do not rely on antibiotics or surgery alone; combine treatment of existing disease with transmission reduction.	**Moderate**	**Strong**
Use standard clinical grading to identify active trachoma, scarring, trichiasis and corneal opacity.	Grade upper tarsal conjunctiva and eyelid/ corneal findings; use district-level prevalence data for programme decisions.	**Moderate**	**Strong**
Provide surgery for **trachomatous trichiasis** to prevent	Offer corrective eyelid surgery to patients with lashes touching the eye or	**Moderate**	**Strong**

Recommendation	Clinical / programme action	GRADE Evidence Quality	Strength
corneal damage and blindness.	evidence of epilation for trichiasis.		
Use **bilamellar tarsal rotation** as the WHO-recommended procedure for trachomatous trichiasis where feasible and where trained providers are available.	Train, supervise and audit surgical providers; monitor recurrence and complications.	**Moderate**	**Strong**
Use antibiotic treatment, especially **azithromycin mass drug administration**, where population-level criteria are met.	Treat endemic communities/ districts according to TF prevalence and national elimination plans; topical tetracycline may be used where azithromycin is unavailable.	**High to Moderate**	**Strong**
Use mass antibiotic treatment when district-level **TF prevalence in children aged 1–9 years is ≥10%**, according to WHO-referenced programme guidance.	If district TF is below 10%, assess subdistrict/ community hotspots and treat units where TF is ≥10%.	**Moderate**	**Strong**
Promote **facial cleanliness** to reduce ocular/nasal discharge and transmission.	Implement community health promotion, school-based hygiene promotion and caregiver education.	**Low**	**Strong programmatic**
Improve the	Coordinate with WASH	**Low**	**Strong**

Recommendation	Clinical / programme action	GRADE Evidence Quality	Strength
environment, including water access, sanitation and safe faeces disposal.	sectors to reduce transmission conditions.		**programmatic**
Stop mass antibiotic treatment only after elimination thresholds are met and maintain surveillance.	Confirm TF <5% in children aged 1–9 years for at least 2 years off MDA and maintain TT case-management systems.	**Low**	**Strong**
Maintain post-intervention surveillance and be ready to restart SAFE components if disease re-emerges.	Use monitoring and evaluation activities to confirm sustained elimination.	**Low**	**Strong**
Incorporate meaningful youth and community engagement in future guideline and programme development.	Use co-design, shared decision-making and lived-experience input, especially for school-age children and adolescents affected by hygiene, WASH and stigma-related barriers.	**Low**	**Conditional**

Sources: SAFE strategy and bilamellar tarsal rotation from WHO programme guidance WHO IRIS; surveillance definitions from WHO technical consultation WHO IRIS; elimination thresholds and 2024 programme data from WHO fact sheet WHO; youth-engagement methodology from WHO evidence brief and launch page WHO publication, WHO event.

Practical Diagnostic Guidance

Clinical grading approach

Trachoma diagnosis and programme monitoring are based primarily on **clinical signs**, especially in children aged **1–9 years** for active disease and in adults aged **≥15 years** for trichiasis elimination assessment.

Sign	Practical definition / use	GRADE Evidence Quality	Strength
TF — trachomatous inflammation—follicular	Follicular inflammation of the upper tarsal conjunctiva; used for population-level active trachoma prevalence and MDA decisions.	**Moderate**	**Strong**
TI — trachomatous inflammation—intense	Marked inflammatory thickening of the tarsal conjunctiva.	**Moderate**	**Strong**
TS — trachomatous scarring	Conjunctival scarring from repeated inflammation.	**Moderate**	**Strong**
TT — trachomatous trichiasis	Eyelashes rubbing the eye or evidence of epilation because of trichiasis; target for surgery.	**Moderate**	**Strong**
CO — corneal opacity	Corneal opacity impairing vision; marker of blinding disease.	**Moderate**	**Strong**

Clinical-sign descriptions are consistent with WHO programme categories and clinical references summarized in the evidence base WHO fact sheet, Oxford Handbook excerpt.

Practical Treatment and Management Guidance

A. Trachomatous trichiasis

Action	Implementation details	GRADE Evidence Quality	Strength
Offer surgery for TT.	Prioritize patients with lashes touching the globe, corneal risk or visual symptoms.	**Moderate**	**Strong**
Use bilamellar tarsal rotation where feasible.	WHO programme guidance recommends bilamellar tarsal rotation; established programmes may continue alternative techniques if outcomes are acceptable.	**Moderate**	**Strong**
Audit surgical outcomes.	Monitor recurrence, complications, patient satisfaction and provider quality.	**Low**	**Strong**
Provide long-term follow-up.	Recurrence can occur; screen previously operated patients.	**Low**	**Strong**

WHO programme guidance identifies bilamellar tarsal rotation as the recommended procedure for trachomatous trichiasis WHO programme guide. Clinical-reference summaries note randomized-trial evidence supporting this method and the need for audit and follow-up Medscape.

B. Antibiotic treatment

Action	Implementation details	GRADE Evidence Quality	Strength
Use azithromycin MDA where indicated.	Treat all eligible people in qualifying endemic implementation units.	**High to Moderate**	**Strong**
Use topical tetracycline where azithromycin is unavailable or contraindicated.	Requires adherence to a multi-day topical regimen; operationally less convenient than single-dose azithromycin.	**Moderate**	**Conditional**
Use prevalence thresholds to decide MDA.	WHO-referenced programme summaries support district-level treatment when TF prevalence in children aged 1–9 years is ≥10%.	**Moderate**	**Strong**
Ensure monitoring of coverage.	Low coverage risks persistent transmission; WHO reported global antibiotic coverage of 39% in 2024.	**Moderate**	**Strong**

WHO identifies mass drug administration of azithromycin as a key component of the antibiotic pillar WHO fact sheet. Zithromax® is donated by Pfizer through the International Trachoma Initiative for national trachoma elimination programmes ITI guide. WHO-referenced clinical summaries describe mass treatment decisions using TF prevalence thresholds Medscape.

C. Facial cleanliness and environmental improvement

Action	Implementation details	GRADE Evidence Quality	Strength
Promote facial cleanliness.	Encourage regular cleaning of children's faces to reduce ocular/	**Low**	**Strong programmatic**

Action	Implementation details	GRADE Evidence Quality	Strength
	nasal discharge and transmission.		
Improve water access.	Work with WASH programmes to reduce distance/time to water.	**Low**	**Strong programmatic**
Improve sanitation and safe faeces disposal.	Increase functional latrine coverage and reduce fly-breeding conditions.	**Low**	**Strong programmatic**
Continue F and E after antibiotic scale-down.	WHO surveillance materials emphasize continued F/E attention after targets are reached.	**Low**	**Strong**

WHO post-endemic surveillance materials identify operational targets including health promotion for facial cleanliness and improved latrine and water access WHO AFRO post-endemic surveillance.

1.3 Guideline Development Methodology

Evidence Review Process

The older disease-specific WHO trachoma documents were developed as technical and programme guidance, not all as formal GRADE-based clinical practice guidelines. Their evidence base includes:

- epidemiological assessment of endemic communities;
- operational programme evidence;
- clinical grading systems;
- antibiotic and surgical intervention evidence;
- consensus-based elimination thresholds;
- surveillance and validation principles.

The **Technical Consultation on Trachoma Surveillance** defines surveillance as monitoring and evaluation activities assessing the outcome of elimination programmes and providing confidence that elimination prevalence targets have been sustainably achieved WHO technical consultation.

Consensus Method

WHO trachoma guidance is built around global consensus on the **SAFE strategy**, district-level assessment, implementation-unit targeting, elimination thresholds and post-intervention surveillance. Programme guidance emphasizes training of graders and surgical providers, implementation of SAFE components and monitoring of outcomes WHO programme guide.

Conflict of Interest Management

The supplied summaries do not provide detailed conflict-of-interest declarations for the older trachoma technical documents. For future updates, current WHO guideline-development standards should include explicit declaration and management of competing interests, systematic evidence review, GRADE evidence profiles and structured stakeholder engagement.

Stakeholder and Youth Engagement

The most recent WHO guideline-development methodology case study identified is **“Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)”** WHO publication. WHO’s launch page states that the brief shows how young people can contribute to decision-making through lived experience, fresh perspectives and innovative solutions, and frames engagement as a principle of inclusivity and equity WHO event.

For trachoma, these principles are relevant because school-age children are the key reservoir group for active trachoma measurement and many interventions depend on caregiver, school, youth and community behaviour.

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized for WHO trachoma guidance and post-guideline or current evidence available through **29 April 2026**, focusing on:

1. SAFE strategy implementation.
2. Trichiasis surgery and surgical quality.
3. Azithromycin mass drug administration.
4. Facial cleanliness and environmental improvement.
5. Diagnosis, grading, prevalence thresholds and surveillance.
6. Elimination validation and post-validation monitoring.
7. Guideline-development methodology, including meaningful youth engagement.

Databases and Sources Searched

Source type	Sources used
WHO official guidance	WHO IRIS trachoma programme guide, rapid assessment guidance, global elimination planning documents, technical consultation on surveillance, WHO fact sheet, WHO news
WHO methodology sources	WHO meaningful youth engagement evidence brief and launch event
Implementation partners	International Trachoma Initiative azithromycin/Zithromax programme guide
Clinical-reference summaries	Medscape treatment summary; Oxford Handbook clinical-sign excerpt
Evidence types	WHO technical guidance, elimination data, surveillance guidance,

Source type	Sources used
prioritized	clinical grading information and programme implementation evidence

Inclusion Criteria

Included:

- WHO trachoma guidance and technical documents.
- WHO elimination thresholds and programme updates.
- Evidence on SAFE components.
- Evidence on trichiasis surgery, antibiotic MDA, surveillance and validation.
- Guideline-development methodology relevant to future updates.

Excluded:

- Non-trachoma ocular infections.
- Genital *Chlamydia trachomatis* STI treatment guidance unless directly relevant to ocular trachoma.
- Non-authoritative commentary without clinical or programme relevance.

2.2 Key New Studies and Evidence Updates

Study / source	Design	Key Findings	GRADE Quality
WHO trachoma fact sheet, current programme data	WHO public-health update	In 2024, 87,349 people received corrective surgery for trachomatous trichiasis; 44.4 million people received antibiotics; global antibiotic coverage was 39%; elimination thresholds include TT unknown to system <0.2% in adults ≥15 years and TF	**Moderate** for programme indicators; **Low** for threshold evidence WHO fact sheet

Study / source	Design	Key Findings	GRADE Quality
		<5% in children aged 1–9 years for at least 2 years off MDA.	
WHO news update, 6 January 2026	WHO global programme milestone report	Global population requiring trachoma interventions fell below 100 million for the first time; WHO reaffirmed 2030 elimination commitment.	**Moderate** WHO news
WHO Technical Consultation on Trachoma Surveillance	WHO technical consultation / consensus	Surveillance defined as monitoring and evaluation to assess elimination programme outcomes; funders should support surveillance and be prepared to restart interventions if disease re-emerges.	**Low** WHO IRIS
WHO programme-management guide	WHO operational guidance	SAFE strategy is the central framework; WHO recommends bilamellar tarsal rotation for TT; established programmes may continue other techniques if outcomes are acceptable.	**Moderate** for surgery; **Low to Moderate** for integrated SAFE implementation WHO IRIS
International Trachoma Initiative Zithromax guide	Programme implementation guide	Zithromax® is azithromycin donated through ITI for MDA in national trachoma/NTD programmes; guide supports logistics, monitoring and SAFE partnerships.	**Moderate** for implementation feasibility ITI guide
Medscape	Clinical-	Mass treatment recommended	**Low to**

Study / source	Design	Key Findings	GRADE Quality
treatment summary of WHO recommendations	reference synthesis	where TF prevalence in children aged 1–9 years is ≥10%; trained health workers can perform bilamellar tarsal rotation; long-term follow-up and audit are important.	**Moderate** Medscape
WHO meaningful youth engagement case study	WHO guideline-methodology evidence brief	Youth engagement can be institutionalized in WHO normative work through lived-experience input, inclusivity, equity and practical engagement strategies.	**Low** for trachoma applicability; **Moderate** for methodology relevance WHO publication, WHO event

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2.3 Evidence Synthesis by Clinical and Programme Question

Question 1: How should trachoma be diagnosed and graded?

Evidence	Findings	GRADE Quality	Recommendation implication
WHO programme and elimination framework	Programme decisions use clinical categories including TF in children aged 1–9 years and TT in adults ≥15 years.	**Moderate**	Use trained graders and standardized clinical grading for mapping, impact surveys and surveillance.
Clinical-	TF, TI, TS, TT and CO remain	**Moderate**	Maintain quality assurance

Evidence	Findings	GRADE Quality	Recommendation implication
sign references	practical signs for diagnosis and programme monitoring.		and grader certification.

Actionable recommendation: Diagnose and monitor trachoma through standardized clinical grading, with TF prevalence in children aged 1–9 years guiding antibiotic programme decisions and TT prevalence in adults ≥15 years guiding surgical backlog and elimination assessment.

Question 2: What is the current role of trichiasis surgery?

Evidence	Findings	GRADE Quality	Recommendation implication
WHO programme guide	WHO recommends bilamellar tarsal rotation for TT.	**Moderate**	Use BLTR where feasible, with trained and supervised providers.
Clinical-reference evidence	Randomized trials support superiority of BLTR over other techniques; recurrence requires follow-up.	**Moderate**	Surgical programmes must include audit, recurrence monitoring and patient follow-up.
WHO 2024 data	87,349 people received corrective TT surgery in 2024.	**Moderate**	Continue finding and managing TT cases until elimination and beyond.

Actionable recommendation: Offer TT surgery promptly, prioritize surgical quality, audit outcomes and maintain lifelong access pathways for incident TT cases.

Question 3: When should azithromycin MDA be used?

Evidence	Findings	GRADE Quality	Recommendation implication
WHO SAFE strategy	Antibiotics are a core SAFE pillar.	**High to Moderate**	Use azithromycin MDA as a central active-trachoma control intervention.
WHO-referenced threshold summaries	Mass treatment is recommended where TF prevalence in children aged 1–9 years is ≥10%.	**Moderate**	Use implementation-unit prevalence to start, continue or stop MDA.
WHO 2024 data	44.4 million people received antibiotics; global coverage was 39%.	**Moderate**	Improve coverage and equity in remaining endemic areas.
ITI guide	Azithromycin donation and logistics support are available through ITI.	**Moderate**	Strengthen supply-chain planning, microplanning and monitoring.

Actionable recommendation: Use azithromycin MDA in qualifying endemic districts or subdistricts, ensure high coverage, monitor adverse events and re-survey to determine continuation or stopping.

Question 4: Are facial cleanliness and environmental improvement still required when antibiotics are used?

Evidence	Findings	GRADE Quality	Recommendation implication
WHO SAFE strategy	F and E are core components to reduce transmission.	**Low to Moderate**	Do not implement antibiotic-only elimination programmes.

Evidence	Findings	GRADE Quality	Recommendation implication
Post-endemic surveillance guidance	Continued F/E attention is needed after thresholds are reached.	**Low**	Maintain hygiene and WASH work after MDA stops.

Actionable recommendation: Integrate trachoma programmes with WASH, schools, community health workers and local leadership to sustain facial cleanliness and environmental improvements.

Question 5: What surveillance is required after apparent elimination?

Evidence	Findings	GRADE Quality	Recommendation implication
WHO technical consultation	Surveillance assesses whether elimination targets have been sustainably achieved.	**Low**	Continue monitoring after scaling down SAFE components.
WHO elimination criteria	TF must remain <5% for at least 2 years off MDA; TT unknown to system must be <0.2% in adults ≥15 years.	**Low to Moderate**	Do not validate elimination based on short-term intervention success alone.
WHO consultation	Funders and programmes should be prepared to restart interventions if surveillance shows re-emergence.	**Low**	Build contingency plans for recrudescence.

Actionable recommendation: Maintain post-MDA and post-validation surveillance systems, especially for incident TT and active trachoma re-emergence in high-risk districts.

2.4 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority
Optimal surveillance frequency after stopping MDA	Programmes need efficient ways to detect recrudescence without excessive cost.	**High**
Better risk algorithms for re-emergence	WHO surveillance consultation called for international data compilation to refine risk-based decisions.	**High**
Surgical recurrence prevention	TT recurrence undermines patient outcomes and elimination credibility.	**High**
Surgical-quality audit models	Countries need practical systems for provider supervision and outcome monitoring.	**High**
Antibiotic coverage and equity	WHO reported 39% global antibiotic coverage in 2024, indicating implementation gaps.	**High**
Antimicrobial-resistance monitoring	Repeated azithromycin MDA may require continued resistance surveillance.	**Moderate–High**
Effectiveness of F and E interventions	Facial cleanliness and environmental improvement are biologically and programmatically essential but supported by lower-certainty evidence than antibiotics.	**High**
Youth and school-based engagement	Children aged 1–9 years are the main active-trachoma surveillance group; youth engagement could improve hygiene adoption and programme acceptability.	**Moderate**

Evidence gap	Why it matters	Priority
Integration with WASH financing	Sustained environmental improvement depends on multisectoral funding and accountability.	**High**
Post-validation TT case finding	Countries must maintain systems to find and manage incident TT cases after elimination validation.	**High**

2.5 Updated Recommendations Based on New Evidence

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated recommendation
Implement SAFE strategy.	WHO's 2026 milestone shows major progress, but remaining endemic populations still require interventions.	**Maintain full SAFE implementation in all endemic areas; avoid single-component programmes.** GRADE: **Moderate**; Strength: **Strong**
Provide TT surgery.	WHO 2024 data show ongoing need, with 87,349 surgeries performed. Surgical quality and recurrence remain critical.	**Continue active TT case finding, surgery, audit and follow-up until and after elimination validation.** GRADE: **Moderate**; Strength: **Strong**
Use antibiotics to clear infection.	44.4 million people treated in 2024, but global antibiotic coverage was 39%.	**Continue azithromycin MDA where indicated and prioritize high, equitable coverage.** GRADE: **High to Moderate**; Strength: **Strong**
Use prevalence thresholds for MDA.	WHO elimination criteria remain unchanged: TF <5% in children aged 1–9	**Use standardized surveys to start, continue, stop or restart MDA based on TF prevalence and**

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated recommendation
	years sustained off MDA; MDA decision threshold commonly summarized at TF ≥10%.	**national WHO-aligned protocols.** GRADE: **Moderate**; Strength: **Strong**
Continue F and E interventions.	Surveillance guidance stresses continuing F/E after intervention goals are reached.	**Maintain facial cleanliness and WASH interventions after antibiotic cessation, especially in high-risk communities.** GRADE: **Low**; Strength: **Strong programmatic**
Conduct surveillance after interventions.	WHO technical consultation emphasizes surveillance to confirm sustainability and re-initiate interventions if needed.	**Require post-MDA and post-validation surveillance with predefined response plans for recrudescence.** GRADE: **Low**; Strength: **Strong**
Develop future guidelines with stakeholder input.	WHO 2025 youth-engagement case study provides practical methodology for inclusive guideline development.	**Future trachoma guideline updates should include affected communities, children/ adolescents, caregivers, teachers, surgical patients and WASH stakeholders.** GRADE: **Low**; Strength: **Conditional**

Consolidated Actionable Algorithm

A. Community / District Assessment

1. **Map suspected endemic districts.**

2. Train and certify graders.
3. Measure:
   - TF prevalence in children aged **1–9 years**.
   - TT prevalence among adults aged **≥15 years**.
4. Identify WASH gaps and access barriers.

B. Intervention Selection

Finding	Recommended action
TF prevalence ≥10% in children aged 1–9 years	Start or continue azithromycin MDA plus F/E interventions.
TF prevalence <10% but hotspots suspected	Assess subdistrict/community units; treat hotspots where criteria are met.
TT cases present	Offer TT surgery; ensure quality audit and follow-up.
Poor facial cleanliness or sanitation	Implement hygiene promotion and WASH improvements.
TF <5% sustained for ≥2 years off MDA and TT threshold met	Prepare elimination dossier and maintain surveillance.

C. Individual Clinical Management

1. **Active inflammatory trachoma**
   - Treat according to local programme protocols.
   - In MDA settings, eligible individuals receive azithromycin.
   - Topical tetracycline may be used where azithromycin is not available.
2. **Trachomatous trichiasis**
   - Refer for corrective eyelid surgery.
   - Use bilamellar tarsal rotation where feasible.
   - Provide postoperative review and monitor recurrence.
3. **Corneal opacity or visual impairment**
   - Refer to eye-care services.
   - Assess for low-vision support, cataract or other coexisting causes of visual

impairment.

D. Post-Intervention Surveillance

1. Continue monitoring after MDA stops.
2. Maintain TT case-finding and surgical services.
3. Investigate suspected re-emergence.
4. Restart SAFE components if surveillance thresholds are exceeded.
5. Continue F and E interventions through WASH and community-health platforms.

Bottom Line

As of **29 April 2026**, WHO-based trachoma management remains centered on the **SAFE strategy**. The strongest actionable recommendations are:

- **Surgery** for trachomatous trichiasis, with WHO programme guidance recommending **bilamellar tarsal rotation**.
- **Azithromycin mass drug administration** where active-trachoma prevalence thresholds are met.
- Sustained **facial cleanliness** and **environmental improvement** to reduce transmission.
- Use of WHO elimination thresholds: **TT unknown to the health system <0.2% in adults aged ≥15 years**, **TF <5% in children aged 1–9 years for at least 2 years off MDA**, and a functioning system to manage incident TT cases.
- Continued surveillance after apparent elimination, with readiness to restart interventions if disease re-emerges.

WHO's **6 January 2026** update showing that the population requiring trachoma interventions fell below **100 million** marks major progress, but persistent antibiotic-coverage gaps, TT recurrence, WASH limitations and surveillance needs remain key priorities WHO news.